

COOK COUNTY PEDIATRIC SPECIALTY CLINICS

Division of Pediatric Ophthalmology

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PEDIATRIC OPHTHALMOLOGY NOTE

Patient: Marcus D. Whitfield MRN: PED-2025-04471

DOB: 03/03/2016 Age: 9 years, 1 month

Date of Visit: 04/15/2025 Visit Type: Follow-up

Attending: Diana Reyes, MD — Pediatric Ophthalmology

Accompanied By: Foster parent (prior placement)

REASON FOR VISIT

Follow-up for esotropia and amblyopia management.

HISTORY

Marcus has been followed for intermittent esotropia since age 5, first identified during a preschool vision screening. He completed a course of orthoptic exercises (vision therapy) at our center, finalized in 09/2023, with partial improvement in binocular alignment. Glasses were prescribed at the previous visit for hyperopic correction to address the accommodative component of his esotropia. The caregiver reports that Marcus lost his glasses approximately 2 months ago during a foster placement transition, and replacement has not been arranged. She notes that he squints frequently and tilts his head to the left when trying to focus on near work.

MEDICATIONS RELEVANT TO OPHTHALMOLOGY

Risperidone 1.5 mg QHS (no known ocular side effects at current dose). No topical ophthalmic medications.

EXAMINATION

Visual Acuity (uncorrected): OD 20/40, OS 20/30. Cover Test: Intermittent esotropia, 15 prism diopters at distance, 20 PD at near. Cycloplegic Refraction: OD +1.25, OS +2.00. Pupils: Equal, round, reactive to light and accommodation. No afferent pupillary defect. Extraocular Movements: Full, no restriction, no nystagmus.

Anterior Segment: Clear cornea OU. Anterior chambers deep and quiet. Lenses clear. Intraocular Pressure: 14 mmHg OD, 13 mmHg OS (Tonopen). Fundus (dilated): Normal optic disc appearance bilaterally — no pallor, no edema, no cupping beyond physiologic limits. Cup-to-disc ratio 0.3 OU. Macula: normal foveal reflex OU. Peripheral retina: unremarkable bilaterally.

ASSESSMENT

1. Accommodative esotropia — worsened since glasses were lost. The accommodative component responds to hyperopic correction; without glasses, the uncorrected refractive error is driving increased convergence. New glasses prescription is required urgently to prevent further amblyopia progression.
2. Amblyopia, OD — visual acuity 20/40 uncorrected, likely improvable with consistent optical correction and continued patching therapy. Patching regimen (2 hours daily, OS patched) should continue.
3. History of completed orthoptic therapy — esotropia angle improved from 25 PD to 18 PD at near per prior records. Continued monitoring recommended.

PLAN

- New glasses prescription provided to caregiver: OD +1.25, OS +2.00. Caregiver instructed to obtain glasses as soon as possible. Consistent wear during all waking hours is essential.
- Continue patching OS 2 hours daily for amblyopia treatment.
- Follow-up in 6 months for esotropia monitoring and amblyopia assessment.

Diana Reyes, MD

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Electronically signed: 04/15/2025 at 14:20 CST

